

Mental health care: from declarations to action

In 2023, 15% of the world's population had mental health disorders, with depressive and anxiety disorders being some of the most burdensome. Estimates suggest that if optimal treatments were available, 71% of the global anxiety disorder burden could be avoided. To address this access gap, integration of mental health care into primary care services is widely supported. In September, 2025, world leaders expressed support for the UN global political declaration on non-communicable diseases and mental health, recognising the need to shift mental health care and resources from specialised institutions to services delivered in the community; scale-up the accessibility, availability, and provision of psychosocial and psychological support and of pharmacological treatment; and increase the number, capacity, retention, and skills of health-care workers.

Despite governmental support for integrated mental health care, progress towards targets agreed in the WHO Comprehensive Mental Health Action Plan 2013–2030 remains insufficient. According to the WHO Mental Health Atlas 2024, only 17 (12%) of 138 responding countries met all five WHO criteria for functional integration of mental health care into primary care (ie, guidelines available and implemented nationally, pharmacological treatment available, psychosocial interventions provided in primary care, primary care staff trained to manage mental health conditions, and involvement of mental health specialists in primary care). However, despite slow progress, there are examples of effective integration of mental health care into primary care. In some countries, such as Australia, mental health care is integrated into routine general practice consultations, with community nurses, psychologists, and psychiatrists giving advice and support. In other countries, such as Iran, community health workers have clearly defined responsibilities for mental health care, including active case finding and referral, with mental health specialists and general practitioners also involved in care. Across regions, for example Latin America, there are often a range of programmes, with providers most commonly being teams that include community health or primary care workers and mental health specialists. Implementation differs between contexts due to available resources, differences in digitalisation of health systems, the number of

professionals available in a given country, and more. Hence, integrated mental health care must be both culturally sensitive and tailored to specific local contexts. The integration of mental health care into primary care looks different globally and, despite the ongoing efforts of primary care staff, is not consistently implemented or integrated.

To improve integration, WHO Europe provided four key strategies to scale-up mental health services in primary care. First, enhancing the mental health capacities of primary care workers, training them to recognise early symptoms and provide support. Second, expanding primary care teams with dedicated mental health workers, such as psychologists. Third, strengthening the connection between primary care and specialist mental health services, to provide users with more options and to ease transitions. Fourth, promoting coordination and partnerships with other sectors, including non-medical community services such as housing, education, and employment. However, even in high-income countries, the most disadvantaged areas have fewer general practitioners, clinical primary care staff, and non-clinical primary care staff, despite increased health needs. This disparity in service availability highlights that integration of mental health care into primary care should be equitable, following the principles of universal health coverage. Moreover, interventions to strengthen the quality and accessibility of primary care services, including mental health care, need sustained investment and long-term national implementation plans.

Integration of mental health care into primary care has potential for the quality and accessibility of care. For example, community services can reduce stigma and provide a supportive and understanding environment while also focusing on prevention, which is crucial for mental health conditions. Tailored mental health care that is adapted to different contexts, vulnerable populations, social services available, and local teams can more effectively suit local needs. Without investment in the primary care workforce, the vision of the UN declaration cannot be implemented. ■ *The Lancet Primary Care*

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For 2023 data see <https://www.healthdata.org/research-analysis/health-topics/mental-health-our-approach>

For the UN global political declaration see <https://www.who.int/news/item/26-09-2025-world-leaders-show-strong-support-for-political-declaration-on-noncommunicable-diseases-and-mental-health>

For the WHO Comprehensive Mental Health Action Plan 2013–2030 see <https://www.who.int/publications/i/item/9789240031029>

For the WHO Mental Health Atlas 2024 see <https://iris.who.int/server/api/core/bitstreams/5897b3c7-2848-47a7-ba22-0a7902342a81/content>

For Australian mental health care in primary care see <https://www.racgp.org.au/advocacy/position-statements/view-all-position-statements/clinical-and-practice-management/mental-health-care-in-general-practice>

For Iranian mental health care in primary care see <https://iris.who.int/server/api/core/bitstreams/385a1cb0-1bed-4ece-8045-845584249f3b/content>

For Latin American mental health care in primary care see *Lancet Reg Health Am* 2025; 41: 100931

For the WHO Europe four key strategies see <https://www.who.int/europe/news/item/23-04-2025-scale-up-mental-health-care-within-primary-health-care-who-research-suggests>